Materials Design Analysis Reporting (MDAR) Checklist for Authors

The MDAR framework establishes a minimum set of requirements in transparent reporting applicable to studies in the life sciences (see Statement of Task: doi:10.31222/osf.oio/9sm4x.). The MDAR checklist is a tool for authors, editors and others seeking to adopt the MDAR framework for transparent reporting in manuscripts and other outputs. Please refer to the MDAR Elaboration Document for additional context for the MDAR framework.

Materials

Antibodies	Yes (indicate where provided: page no/section/legend)	n/a
For commercial reagents, provide supplier name, catalogue number and RRID, if available	No antibodes detected. Please add identifiers for all resources where possible	

Cell Materials	Yes (indicate where provided: page no/section/legend)	n/a
Cell lines: Provide species information, strain. Provide accession number in repository OR supplier name, catalog number, clone number, OR RRID	No cell lines detected Please add identifiers for all resources where possible	
Primary cultures: Provide species, strain, sex of origin, genetic modification status.	Not currently checked by SciScore	

Experimental Animals	Yes (indicate where provided: page no/section/legend)	n/a
Laboratory animals: Provide species, strain, sex, age, genetic modification status. Provide accession number in repository OR supplier name, catalog number, clone number, OR RRID	No organisms detected Please add identifiers for all resources where possible	
Animal observed in or captured from the field: Provide species, sex and age where possible	Not currently checked by SciScore	
Model organisms: Provide Accession number in repository (where relevant) OR RRID	See laboratory animals section for information.	

Plants and microbes	Yes (indicate where provided: page no/section/legend)	n/a
Plants: provide species and strain, unique accession number if available, and source (including location for collected wild specimens)	Not currently checked by SciScore	
Microbes: provide species and strain, unique accession number if available, and source	Not currently checked by SciScore	

Human research participants	Yes (indicate where provided: page no/section/legend)	n/a
Identify authority granting ethics approval (IRB or equivalent committee(s), provide reference number for approval.	Ethics This study complies with the Declaration of Helsinki and was approved by the Ethics Committee of the Medical Faculty of the University of Wurzburg (17613).	
Provide statement confirming informed consent obtained from study participants.	Inclusion criteria were the diagnosis of acute IS according to the World Health Organization definition, 18 age >= 18 years, and provision of informed consent.	
Report on age and sex for all study participants.	**Age:**Inclusion criteria were the diagnosis of acute IS according to the World Health Organization definition, 18 age >= 18 years, and provision of informed consent. **Sex:**17 Table 1 Definitions of systolic dysfunction , diastolic dysfunction , and heart failure Systolic dysfunction 17 LVEF < 52 (men) and <54 (women) Diastolic dysfunction (>=3 criteria fulfilled in patients without systolic dysfunction) 16 LAVI > 34 mLm2 OR LA area > 30 cm2 (if LAVI not available) Average Ee > 14 Lateral e < 10 cms OR septal e < 7 cms Tricuspid regurgitation maximal flow velocity > 2.8 ms Heart failure 15 Heart failure with preserved ejection fraction Heart failure with mid-range ejection fraction Heart failure with reduced ejection fraction LVEF >= 50 LVEF	

Human research participants	Yes (indicate where provided: page no/section/legend)	n/a
	40-49 LVEF < 40 At least one of the following: LAVI > 34 mLm2	

Design

Study protocol	Yes (indicate where provided: page no/section/legend)	n/a
For clinical trials, provide the trial registration number OR cite DOI in manuscript.	Not detected.	

Laboratory protocol	Yes (indicate where provided: page no/section/legend)	n/a
Provide DOI or other citation details if detailed step-by-step protocols are available.	Not detected.	

Experimental study design (statistics details)	Yes (indicate where provided: page no/section/legend)	n/a
State whether and how the following have been done, or if they were not carried out		
Sample size determination	Sample size calculation was based on previously published estimates of 10-24 . 4 , 5 , 7 , 8 We aimed to recruit 750 patients to reach prevalence estimates of 10 with a precision of 8.1-24 or of 24 with a precision of 21.1-27.2 .	
Randomization	not detected.	
Blinding	25 Measurements were performed by an operator blinded to all clinical data .	
inclusion/exclusion criteria	Inclusion criteria were the diagnosis of acute IS according to the World Health Organization definition, 18 age >= 18 years, and provision of informed consent.	

Sample definition and in-laboratory replication	Yes (indicate where provided: page no/section/legend)	n/a
State number of times the experiment was replicated in laboratory	Not detected.	
Define whether data describe technical or biological replicates	Not detected.	

Ethics	Yes (indicate where provided: page no/section/legend)	n/a
Studies involving human participants: State details of authority granting ethics approval (IRB or equivalent committee(s), provide reference number for approval.	Ethics This study complies with the Declaration of Helsinki and was approved by the Ethics Committee of the Medical Faculty of the University of Wurzburg (17613).	
Studies involving experimental animals: State details of authority granting ethics approval (IRB or equivalent committee(s), provide reference number for approval.	Not detected.	
Studies involving specimen and field samples: State if relevant permits obtained, provide details of authority approving study; if none were required, explain why.	Not detected.	

Dual Use Research of Concern (DURC)	Yes (indicate where provided: page no/section/legend)	n/a
If study is subject to dual use research of concern, state the authority granting approval and reference number for the regulatory approval	Not currently checked by SciScore	

Analysis

Attrition	Yes (indicate where provided: page no/section/legend)	n/a
State if sample or data point from the analysis is excluded, and whether the criteria for exclusion were determined and specified in advance.	25 Measurements were performed by an operator blinded to all clinical data .	

Statistics	Yes (indicate where provided: page no/section/legend)	n/a
Describe statistical tests used and justify choice of tests.	Statistical analysis According to the distribution of the variables , differences between groups were tested using the kh 2 test , Students t-test , ANOVA , Mann-Whitney U test , or Kruskal-Wallis test .	

Data availability	Yes (indicate where provided: page no/section/legend)	n/a
State whether newly created datasets are available, including protocols for access or restriction on access.	Not detected.	
If data are publicly available, provide accession number in repository or DOI or URL.	Not detected.	
If publicly available data are reused, provide accession number in repository or DOI or URL, where possible.	Not detected.	

Code availability	Yes (indicate where provided: page no/section/legend)	n/a
For all newly generated code and software essential for replicating the main findings of the study:		
State whether the code or software is available.	Not detected.	
If code is publicly available, provide accession number in repository, or DOI or URL.	Not detected.	

Analysis

Adherence to community standards	Yes (indicate where provided: page no/section/legend)	n/a
MDAR framework recommends adoption of discipline-specific guidelines, established and endorsed through community initiatives. Journals have their own policy about requiring specific guidelines and recommendations to complement MDAR.		
State if relevant guidelines (eg., ICMJE, MIBBI, ARRIVE) have been followed, and whether a checklist (eg., CONSORT, PRISMA, ARRIVE) is provided with the manuscript.	Not currently checked by SciScore	